AMA
AMERICAN MEDICAL ASSOCIATION

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The Honorable Mehmet C. Oz, MD, MBA
Administrator
Centers for Medicare & Medicaid Services
Hubert H. Humphrey Building
200 Independence Avenue, SW
Washington, DC 20201

Re: RIN 0938-AV68 Health Technology Ecosystem Request for Information

Dear Administrator Oz:

On behalf of the physician and medical student members of the American Medical Association (AMA), I appreciate the opportunity to provide feedback to the Centers for Medicare & Medicaid Services (CMS) on its Request for Information (RFI), Health Technology Ecosystem (CMS-0042-NC). We commend CMS, working in concert with the Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology (ASTP/ONC), for seeking detailed stakeholder input to accelerate secure, patient-centered data exchange and promote digital health tools for Medicare beneficiaries while easing the administrative and financial burdens that hinder physicians' ability to provide care. The AMA has included specific CMS questions and the AMA's responses below.

Patients and Caregivers

1. Patient Needs

PC-1. What health management or care navigation apps would help you understand and manage your (or your loved ones) health needs, as well as the actions you should take?

a. *What are the top things you would like to be able to do for you or your loved ones' health that can be enabled by digital health products?*
b. *If you had a personal assistant to support your health needs, what are the top things you would ask them to help with? In your response, please consider tasks that could be supported or facilitated by software solutions in the future.*

The AMA welcomes CMS' exploration of consumer-facing digital assistants and care-navigation applications (apps) that can handle the administrative chores our patients find most frustrating, coordinating specialist appointments, appealing insurance denials, secure messaging with clinicians, and compiling clear, longitudinal health-record summaries. When designed well, artificial intelligence (AI)-enabled tools can reduce administrative friction for both patients and physicians, streamline access to care, and allow clinicians to focus on medical decision-making rather than paperwork.

To ensure those benefits are realized without eroding trust, CMS should anchor any future policy in six guardrails: **(1) a mandatory "human-override" so a patient or clinician can reverse any AI-generated action; (2) full transparency on an app's training data sources and decision logic; (3) coordination with the U.S. Department of Health and Human Services (HHS) Office for Civil**

Rights (OCR) to enforce the Health Insurance Portability and Accountability Act's (HIPAA) minimum-necessary rule for any disclosures made to a person or entity who is not the patient, and coordination with the ASTP/ONC to extend electronic health record (EHR) transparency attributes to third-party apps; (4) continuous post-deployment surveillance with public reporting of performance, bias, and security events; (5) pre-market safety and usability testing akin to existing health-IT certification, tailored to administrative functions; and (6) meaningful penalties for data misuse, impermissible disclosures, or unremedied security flaws. These safeguards address our patients' top concerns—data breaches, billing errors from faulty insurance data, opaque algorithms, data misuse, and AI interfering with the patient-physician relationship—while preserving the innovation needed to deliver truly helpful digital health assistants.

PC-2(c). Do you have easy access to your own and all your loved ones' health information in one location (for example, in a single patient portal or another software system)? Were there particular data types, such as x-rays or specific test results, that were unavailable? What are the obstacles to accessing your own or your loved ones' complete health information electronically and using it for managing health conditions or finding the best care (for example, limitations in functionality, user friendliness, or access to basic technology infrastructure)?

Our patients value seamless, secure access to a complete longitudinal record. Patients and physicians know first-hand that the roadblocks are largely not created by providers but by fragmented technology and inconsistent developer practices. Separate logins for every portal, office notes released only as low-quality portable document formats (PDF), imaging locked on compact disk read-only memory (CD-ROMs), and inconsistent, non-standardized use of established medical terminology across sites make it nearly impossible to assemble or trust a single source of truth about patients' health. HIPAA and the information-blocking rules promise access, yet without stronger federal oversight of certified health information technology (health IT) developers, those promises remain unfulfilled.

CMS and ASTP/ONC should use every certification and payment lever at their disposal to address this flawed dynamic. Specifically, certified systems should be required to: **(1) deliver imaging in standard fast health care interoperability resources (FHIR)-based digital image and communication in medicine (DICOM) application programming interfaces (API); (2) generate longitudinal records that are de-duplicated and use standardized terminology; (3) face monetary or certification penalties if developers restrict patients to PDFs or CD-ROMs; (4) embed data-labeling and segmentation to protect sensitive information and build trust; (5) give patients the exact data and level of detail provided to clinicians; (6) prohibit price-gouging for simplified patient or digital-identity access; and (7) offer patients and physicians free or low-cost connections to nationwide and state health-information networks such as the Trusted Exchange Framework and Common Agreement (TEFCA), Carequality, and local health information exchanges (HIE).** These steps would finally make "easy access in one place" a reality, empower shared decision-making, and allow clinicians to focus on the chronic-disease crisis instead of chasing records.

PC-5. What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?

a. *What role, if any, should CMS have in reviewing or approving digital health products on the basis of their efficacy, quality or impact or both on health outcomes (not approving in the sense of a coverage determination)? What criteria should be used if there is a review process?*

CMS should endorse only those digital health tools that can prove clinical accuracy, safety, and interoperability. For any product that relies on patient-generated data, such as home blood-pressure cuffs, glucometers, or pulse oximeters, validation must follow recognized testing protocols like those used for the AMA Validated Device Listing (www.validatebp.org). We urge CMS to convene an independent, multi-stakeholder advisory panel to establish clear approval criteria: documented improvement in the intended patient population, user-friendly design, HIPAA-grade privacy and security, and FHIR-based data exchange. **Digital tools that meet this bar should be fast-tracked for Medicare coverage and patient-education campaigns; those that do not, should not carry any CMS endorsement.** This approach builds patient trust while giving clinicians confidence that device data can safely inform care.

PC-6. What features are most important to make digital health products accessible and easy to use for Medicare beneficiaries and caregivers, particularly those with limited prior experience using digital tools and services?

Accessible digital-health tools require three things: reliable connectivity, clear instructions, and device features that synchronize seamlessly with clinical workflows.

a. **Close the access gap.** Many beneficiaries still lack affordable broadband; CMS should expand subsidy and infrastructure programs so medical-grade devices can transmit data without interruption.
b. **Focus on clinically actionable devices.** Tools that capture medical metrics—blood pressure (BP), glucose, pulse oximetry, heart-rhythm strips—drive real care decisions, unlike step counters. CMS should prioritize coverage for these devices (e.g., home BP cuffs for all hypertensive beneficiaries, not just end stage renal disease patients).
c. **Keep the user experience simple.** Devices should offer Bluetooth or cellular auto-upload, require minimal setup, and feed data directly into the EHR. CMS should provide plain-language guides and short videos, such as those already in use for home blood-pressure monitoring (see AMA self-monitored blood pressure (SMBP) video and ValidateBP resources).[1] Websites and portals must use large fonts, high contrast, and straightforward "share with my doctor" buttons to support seniors and novice users.

[1] https://map.ama-assn.org/resources/self-measured-blood-pressure-smbp-training-video; https://www.validatebp.org/; https://map.ama-assn.org/sites/default/files/2025-03/How%20to%20Measure%20Your%20Blood%20Pressure%20at%20Home%20%28English%29.pdf.

2. Data Access and Integration

PC-8. In your experience, what health data is readily available and valuable to patients or their caregivers or both?

c. *What specific opportunities and challenges exist to improve accessibility, interoperability and integration of clinical data from different sources to enable more meaningful clinical research and generation of actionable evidence?*

For most patients and caregivers, the **single most useful—and already widely available—set of clinical data is the Current Procedural Terminology (CPT®)-coded record of procedures, services** that appears in every EHR portal, Explanation-of-Benefits statement, and claims summary. Because CPT is the nation's uniform "common language" for medical services—maintained through an open, evidence-based process led by the AMA—it delivers four things patients say they need: **(1) a concise, plain-language descriptor explaining *what was done*; (2) a reliable bridge to price-estimation tools, since payers base cost-sharing on CPT codes; and (3) drives the identification of patient to receive targeted clinical interventions through innovative, value-based care arrangements.** Importantly, patients never pay to view their CPT-coded information; the descriptors travel with the clinical record at no charge, and their universal use across physicians, hospitals, and payers already underpins care coordination and longitudinal personal-health records.

Opportunities and Challenges.

To unlock even more value for research and "actionable evidence," CMS should **require every certified API to expose FHIR Procedure resources populated with CPT codes**, support services that advance CPT harmonization with systematized nomenclature of medicine clinical terms (SNOMED CT), international classification of diseases (ICD-10), and Healthcare Common Procedure Coding System (HCPCS). These steps would let clinical, claims, and device data line up automatically across settings, powering cohort discovery and AI analytics without reinventing new vocabularies. **The chief challenges are operational: EHR developers must upgrade interfaces, small and medium sized practices need low-cost implementation support, and privacy safeguards must ensure that richer data flows still respect state and federal protections.**

PC-10. How is the Trusted Exchange Framework and Common Agreement™ (TEFCA™) currently helping to advance patient access to health information in the real world?

The AMA appreciates the work of CMS and ASTP/ONC to promote the seamless and secure flow of health information between patients, providers, and payers. We share the objective of advancing data exchange to ensure that physicians have secure access to the right information about the right patient at the right time in their care journey. We see the Trusted Exchange Framework and Common Agreement (TEFCA) as a means to enable enhanced information exchange that should remain as an option on the data exchange landscape.

TEFCA is making significant progress toward ramping up data exchange nationwide. There are 2,816 organizations live on TEFCA—Qualified Health Information Networks (QHINs), participants, sub-participants, and more than 14 million documents have been shared on the network since its go-live date in December 2023.

The nine federally designated QHINs are among the most recognized names in the health IT field. For example, the health IT developer Epic's community members connect to TEFCA through Epic Nexus, where more than 1,000 hospitals and 22,000 clinics using Epic are now live on TEFCA.

Nevertheless, while many physicians report that they can connect to local health information exchange (HIE) networks, they often cannot access their patients' complete health history. **This results in a lack of trust when it appears that important medical information is missing. Physicians will forgo using an HIE if they do not feel they can find and receive a complete patient record.** Furthermore, physicians often experience a unidirectional flow of information. While patient information is often requested from physicians' EHR systems, physicians frequently do not receive information when they make similar requests.

The AMA views TEFCA as an opportunity to better enable all interested parties to access patients' longitudinal health records in a secure environment. **However, we stress that CMS and ASTP/ONC should not make TEFCA participation a requirement for physicians; rather, it should remain as an option among the many means of data exchange currently available.** We have moved well past the belief that physicians cannot or will not adopt or use health IT unless forced or prodded. In contrast, from the AMA's experience, and that of hundreds of thousands of physician members, physicians will voluntarily adopt health IT if provided a clear return on **their** investment. It serves their needs to care for patients and benefits public health.

While TEFCA may expand the availability of medical information, more can be done to improve the usefulness of, and trust in, exchanged information. **Efforts by CMS and ASTP/ONC to increase data exchange among health care stakeholders must ensure that patient data is protected, safe, and secure.** Patients are most comfortable with physicians and hospitals having their data and are least comfortable with their data being exposed outside the provider space.[2] Trust is a fundamental aspect of the patient-physician relationship. Even well-informed and knowledgeable patients rely on their physicians to provide them with appropriate information, keep personal information confidential, and act in their best interests.

The AMA encourages CMS and ASTP/ONC to work across HHS and the entire federal government to promote trust, strengthen data privacy, and facilitate greater data exchange that benefits patients, through TEFCA as well as other means.

3. Information Blocking and Digital Identity

PC-13. How can CMS encourage patients and caregivers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal? What would be the impact? Would increasing reporting of complaints advance or negatively impact data exchange?

The AMA applauds CMS' goal of empowering patients to flag true information-blocking incidents, but success hinges on making the process intuitive and accurately aimed at the real culprits—technology developers that hide behind "technical limitations" and exorbitant interface fees. **CMS and ASTP/ONC should:**

[2] https://www.ama-assn.org/system/files/ama-patient-data-privacy-survey-results.pdf.

- Embed a one-click "Report a Sharing Problem" button in every certified patient portal and Blue Button–enabled apps that route complaints first to the practice (so physicians can clarify or correct the request) and, if unresolved, forwards the report—and relevant log files—directly to ASTP/ONC's portal with the developer clearly identified.
- Require developers to post plain-language, in-portal notices explaining patients' access rights, the difference between physician and developer responsibilities, and step-by-step instructions (including a toll-free hotline with live assistance) for filing a complaint with the developer.
- Launch a CMS-Office for Civil Rights (OCR)-ASTP/ONC public-awareness campaign—short videos in the Medicare Connected Apps Directory, shareable clips on YouTube and social media, and partnerships with national and local patient-advocacy groups—using clear, non-legal language and real-world examples.

Done this way, higher-quality complaints will spotlight systemic developer barriers, guiding ASTP/ONC enforcement and hastening interoperability. **If, however, CMS boosts reporting without these guardrails, the portal will be flooded with misdirected grievances against physicians, eroding patient-clinician trust and wasting oversight resources.** Effective patient outreach paired with vendor accountability will advance data exchange; unfocused complaint volume will not.

PC-14(c)&(e). Regarding digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 credentialing service providers (CSP)), what are the potential downsides? What role should CMS/payers, providers, and app developers have in driving adoption?

Digital-identity credentials could one day spare patients the password chaos of multiple portals and make it easier to share records securely but rolling them out before the underlying risks are fixed will backfire. Centralized biometric hubs—like those some commercial vendors now propose—create a single treasure-trove of immutable data that can be repurposed for surveillance, marketing, or AI training, and once breached, can never be "re-issued." Patients would lose meaningful choice as these systems become embedded in pharmacies, hospitals, and insurers' apps, yet still shoulder the consequences of any data compromise or billing error triggered by an algorithm that misidentifies them. The public's trust is already shaken by reports of high-resolution facial images being used for commercial profiling and by large-scale breaches of genetic and biometric databases; pushing adoption too aggressively will deepen that skepticism and slow, not accelerate, digital-health uptake.

CMS should therefore condition any future incentives on five concrete safeguards: (1) require decentralized, wallet-based credentials stored locally in secure personal apps, with biometric data retained on the user's device rather than in a national database (2) an explicit ban on secondary data use or sale; (3) require mandatory breach notification along with a vendor-funded remediation fund, so patients are not left to deal with the consequences on their own; (4) annual, independent security audits with public "report cards" on privacy, uptime, and complaint volume; and (5) a clear opt-out pathway that guarantees equal access to care for those who choose passwords over biometrics. In this model, health IT and app developers build and certify the technology, payers pilot large-scale deployments, and CMS collaborates with OCR and ASTP/ONC to enforce HIPAA's minimum-necessary standard for any disclosures made to a person or entity who is not the patient and extend EHR transparency attributes to identity tools. Physicians will adopt digital IDs only after the ecosystem proves it can protect patients, integrate seamlessly into clinical workflows, and remain affordable—costs that belong on the vendors' balance-sheet, not on small and medium sized practices already stretched thin.

Providers

1. Digital Health Apps

PR-1. What can CMS and its partners do to encourage providers, including those in rural areas, to leverage approved (see description in PC-5) digital health products for their patients?

The AMA appreciates Abe Sutton noting the potential for the Medicare Diabetes Prevention Program (MDPP) to achieve positive results in the administration's newly revamped strategy for the Centers for Medicare and Medicaid Innovation. However, enrollment for the MDPP stood at just 9,015 Medicare beneficiaries at the end of March 2024, which is striking considering more than half a million participate in the Centers for Disease Control and Prevention's (CDC) National DPP program (NDPP). A major reason for this discrepancy is that there are over 1,500 CDC-recognized organizations offering the CDC's NDPP compared with just 414 suppliers for the MDPP. One primary reason for this is the MDPP limits suppliers to programs with an in-person component, which prevents Medicare beneficiaries from taking advantage of the types of fully virtual programs that have greatly expanded access to the CDC's NDPP.

We therefore urge CMS to align NDPP and MDPP supplier regulatory requirements by removing the unnecessary stipulation that MDPP suppliers have an in-person presence. Doing so will reduce regulatory complexity, promote alignment between two HHS programs, and drastically increase access to the MDPP services, especially in rural, underserved, and other communities that presently lack suppliers of MDPP services.

PR-2. What obstacles that prevent development, deployment, or effective utilization of the most useful and innovative applications for physician workflows, such as quality measurement reporting, clinical documentation, and billing tasks? How could these obstacles be mitigated?

There is an assumption by CMS that if a physician uses certified health record technology (CEHRT) that quality measurement implementation is seamless and guarantees measures are plug-and-play for practices. Unfortunately, that is not the case and requires significant work to build workflows, train staff, and ensure data are captured in discreet fields. Given the time and resources required to implement quality measures using data from EHRs, the current time provided by CMS to implement new or update existing measures is insufficient—while specifications may be released earlier in the year, they are not finalized until November, with implementation starting in January. There is a need for vendors and CMS to prepare systems before practices start reporting and additional lead times to ensure that staff are trained, workflows are created, and data reliability and validity for each practice are established. Therefore, **we recommend CMS provide advanced implementation time for electronic clinical quality measures (eCQM)/digital quality measures (dQM) and EHR integration requirements. Vendors also charge additional fees for quality reporting document architecture (QRDA) 1 files, and those fees can range from $350 to $2,500 per provider per year.**

PR-3. How important is it for healthcare delivery and interoperability in urban and rural areas that all data in an EHR system be accessible for exchange, regardless of storage format (for example, scanned documents, faxed records, lab results, free text notes, structured data fields)? Please address all of the following:

a. *Current challenges in accessing different data formats.*
b. *Impact on patient care quality.*
c. *Technical barriers to full data accessibility.*
d. *Cost or privacy implications of making all data formats interoperable.*
e. *Priority level compared to other interoperability needs.*

Comprehensive access to every data format in an EHR is mission-critical for both urban and rural practices, as unstructured or siloed content routinely delays care, drives up costs, and erodes patient trust. Ambulatory clinics confront several recurring scenarios: **(1) PDFs buried in the chart hiding echocardiogram recommendation, delaying a heart-failure diagnosis and leading to an avoidable admission; (2) Health Level 7 (HL7) v2 lab feeds that drop Logical Observation Identifiers Names and Codes (LOINC) codes force duplicative vitamin-D and ferritin tests; (3) a free-text, scanned discharge summary led to a double-entered opioid dose and an ED near-miss; (4) Digital Imaging and Communications in Medicine (DICOM) studies delivered on compact disks (CD) postponed a vascular procedure by 48 hours; (5) low-resolution faxed prior authorization forms consume two staff-hours per biologic request; and (6) an unlabeled behavioral-health note, auto-shared through the portal, violated patient privacy and undermined mental-health follow-up.**

These examples illustrate: (a) the day-to-day challenge of accessing scanned images, faxes, free text, and legacy HL7 feeds; (b) the direct hit on quality—duplication, diagnostic delay, medication error, and fractured coordination; and (c) the technical barriers of missing codes, absent viewers, lack of optical-character recognition, and no data-segmentation tags. (d) Converting every format into interoperable, privacy-preserving, computable data will require real money and rigorous safeguards, but the alternative is continued waste, safety risk, and rising physician burnout.

CMS and ASTP/ONC should therefore: **(1) mandate that certified health IT deliver imaging via FHIR-based DICOM application programing interfaces (API) and apply US Core Data for Interoperability (USCDI) data classes across all ingestion points; (2) tie certification to de-duplicated, terminology-standardized longitudinal records and penalties for PDF- or CD-only output; (3) require built-in data-labeling/segmentation so sensitive notes stay private; and (4) prohibit price-gouging on interoperability features while funding small-practice technical assistance.** (e) Given the magnitude of the harms above, full-format accessibility sits at the top tier of interoperability priorities—on par with TEFCA connectivity and digital-identity standards—because none of those initiatives will succeed if the underlying clinical content remains locked in image files, faxes, or unlabeled free text.

2. Data Exchange

PR-5. Which of the following FHIR APIs and capabilities do you already support or utilize in your provider organization's systems, directly or through an intermediary? For each, describe the transaction model, use case, whether you use individual queries or bulk transactions, and any constraints:

a. *Patient Access API*
b. *Standardized API for Patient and Population Services*
c. *Provider Directory API*
d. *Provider Access API*
e. *Payer-to-Payer API*
f. *Prior Authorization API*
g. *Bulk FHIR – Do you support Group ID-based access filtering for population-specific queries?*
h. *SMART on FHIR – Do you support both EHR-launched and standalone app access? What does the process for application deployment entail?*
i. *CDS Hooks (for clinical decision support integrations).*

The AMA supports physician organizations' systems implementing PR-5 (a) through (i). However, without an EHR certification requirement, many physicians, especially those in small, under-resourced, and/or rural practices, will not be able to access these new APIs or take advantage of their promising capabilities. Establishing a process to evaluate, test, and certify this new technology will create standard APIs, promote end-to-end interoperability between EHRs and payer systems, reduce PA burdens, and allow for an electronic process that is meaningfully integrated within the EHR workflow. Inclusion of PA APIs in EHR certification is particularly important given the Merit-Based Incentive Payment System (MIPS) electronic prior authorization (PA) Promoting Interoperability measure that goes into effect in reporting year 2027. Without inclusion of PA APIs in certification, physicians could face a situation where they are held accountable and punished under MIPS because they do not have access to the APIs.

In addition, **the AMA encourages ASTP/ONC to collaborate with CMS and require that impacted payers, such as Medicare Advantage Organizations, adopt and use certified payer APIs as a condition of their participation in CMS programs, as well as require conformance to other capabilities such as clinical decision support (CDS) Hooks and Bulk FHIR.** Requiring payers to use certified APIs will ensure the standardized implementation necessary to promote interoperability. Finally, the industry also needs to align on one standardized set of code mapping to ensure consistency and fidelity across all health plans and ensure accuracy and reliability of data furnished by prior authorization APIs.

PR-7. What strategies can CMS implement to support providers in making high-quality, timely, and comprehensive healthcare data available for interoperability in the digital product ecosystem? How can the burden of increasing data availability and sharing be mitigated for providers? Are there ways that workflows or metrics that providers are already motivated to optimize for that could be reused for, or combined with, efforts needed to support interoperability?

The AMA urges CMS and ASTP/ONC to tighten the "three-pillar" foundation of interoperability—validated technical capability, transparent trust frameworks, and uniform data structure—by; **(1) mandating real-world testing that proves certified EHRs can exchange and use structured FHIR-based data "without special effort;" (2) expanding USCDI on a predictable cadence to include cost, administrative, and consumer-friendly terminology elements; and (3) enforcing open, standards-**

conformant APIs free from punitive fees or contractual barriers. Together, these actions would give physicians and patients computable access to the full longitudinal record while preserving data integrity, advancing the 21st Century Cures Act mandate that information be both accessible and clinically actionable.

To curb provider burden, CMS and ASTP/ONC should shift compliance onus upstream to vendors: require EHR developers—not clinicians—to embed new USCDI elements, auto-populate metadata, and surface data through existing workflows (e.g., e-prescribing, quality reporting, and MIPS value-based performance metrics) that physicians already optimize. Aligning interoperability measures with these familiar programs, funding technical assistance for small practices, and allowing structured data captured once to satisfy multiple federal requirements will spare clinicians duplicative clicks while accelerating nationwide data liquidity.

PR-8. What are ways CMS or partners can help with simplifying clinical quality data responsibilities of *providers?*

An ongoing problem for physician quality reporting, whether MIPS Value Pathway (MVP) or traditional MIPS, as well as Medicare Shared Savings Program (MSSP), is the high data completeness threshold CMS requires for successful reporting on a quality measure. The threshold is particularly problematic for physicians who practice at multiple sites of services and report on an eCQM or registry measure (and dQM in the future). There is a lack of understanding about the maturity of health IT standards to seamlessly aggregate data from EHRs or registries. Challenges include lack of agreed upon semantic and syntactic standards, data privacy concerns, and patient misidentification. Many physician practices also lack knowledge on how to access providers' "digital endpoints" to collect the data needed for aggregation.

The high data completeness requirement runs counter to CMS' goal of reducing administrative burden within the MIPS program and CMS has not yet adequately addressed our concerns. Since 2020, CMS has required physicians to successfully report on a quality measure for 70 percent of all eligible patients (otherwise known as the data completeness requirement within the MIPS program). Starting in 2024, CMS increased the data completeness requirement to 75 percent of all eligible patients, and we continue to question the feasibility and necessity for such a high threshold. The challenges will be further exacerbated for participants in the MSSP program since ALL MIPS quality policies now apply to the MSSP quality requirements. Accountable care organizations (ACO) report that they continue to encounter barriers to capturing data from some practices due to issues such as an EHR is unable to produce a QRDA 1 file or small practices continue to operate with paper medical records. **We urge CMS to work with the physician, ACO and the EHR vendor communities to find solutions to these data aggregation problems. Until the technology standards are more mature, CMS should reduce the quality measure data completeness requirement within MIPS/MSSP and delay mandatory eCQM or dQM adoption.**

To justify the increased requirement, it is our understanding that there is a perception within CMS that the reporting rates received for many of the eCQMs within MIPS are 100 percent.[3] This may be the case for physicians who practice at one site of service and bill under a single TIN. However, vendors do not truly understand what is intended with data completeness and therefore the percentage received by CMS does

[3] Center for Medicare and Medicaid Services (CMS). Medicare Shared Savings Program Reporting MIPS CQMs and eCQMs in the Alternative Payment Model Performance Pathway (APP) Guidance. Posted 12/12/2022.

not accurately capture the eligible population for each TIN. Some physicians and almost all ACOs provide services across multiple sites using the same national provider identifier (NPI) or taxpayer identification number (TIN) combination, but not all sites (including across sites of service) may participate in MIPS, the registry, or EHR that the physician opts to use for MIPS reporting. Therefore, vendors or practices are just capturing the cases within a single EHR/site, which appears to be 100 percent, but excluding the eligible encounters from other sites of service. This may be the case for physicians who practice at one site of service and bill under a single TIN.

Therefore, **we request that CMS validate its assumption that it is possible to keep increasing the percentage when interoperability and seamless transfer of data are not yet universally available**. We also request that CMS work with a few registries and practices to compare which patients/data they can capture from the practice and/or EHR against what CMS sees for the TIN or in claims. The analysis should also include data from a few specialties such as gastroenterology or radiology, as well as internal medicine and family physicians.

We offer the following examples to illustrate the issue:

Example 1 - Specialty Practice with Vendor X as their EHR

The specialty practice uses the Vendor X EHR to report their quality measures. Several physicians at the practice also provide care at two local skilled nursing facilities (SNFs). Because one of the SNFs also uses Vendor X and has systems set up to enable data sharing with this TIN, Vendor X can include the data in what is reported for MIPS. The other SNF uses Vendor Y and is unable to share data with the practice. Data sharing roadblocks include lack of agreed upon semantic and syntactic standards, data privacy concerns, and patient misidentification. Many physician practices also lack knowledge on how to access providers' "digital endpoints" to collect the data needed for aggregation. **To be clear, purposeful information blocking is unlikely the cause in this instance. Lack of technical capability and awareness are the main culprits.**

As a result, Vendor X is not aware of how many patients from that SNF could be eligible for the measure, and they do not include the SNF's data from Vendor Y when aggregating the data for MIPS reporting. In addition, the vendor has interpreted the data completeness requirement to mean that they must report all the cases that are captured in the EHR system. Because of this misinterpretation of the data completeness requirements, the vendor reports a data completeness rate of 100 percent while unknowingly omitting the cases from the SNF from the denominator.

Example 2 - MSSP Participants Interpretation of Guidance - ACO A

An ACO has one CEHRT system (Vendor A) used across most participating TINs; however, a small number of the participating TINs are specialty practices and Federally Qualified Health Centers (FQHCs), which use different CEHRT systems (Vendors B-D). The ACO can collect data from all participant TINs on Vendor A so the ACO can aggregate the data and complete patient de-duplication before submitting a file to CMS. The ACO was unable to successfully extract and aggregate the data from the other TINs using Vendor B due to data privacy concerns. In addition, although the ACO practices are using CEHRT (Vendors C and D), some of the systems were only able to produce Quality Reporting Document Architecture (QRDA) III files so they are unable to de-duplicate patients. The ACO is also attempting to use billing claims for those practices that are still on paper. Using all these various methods, the ACO estimates a data completeness rate of at least 75 percent, based on the patient volumes. Here again, unaligned implementation of standards and unique customization choices made by CEHRT impact data completeness.

Example 3 - Interpretation of Guidance - ACO B

An ACO has 10 CEHRT EHR systems used across all participating TINs, including several small practices. The ACO is using an external vendor to assist with the data aggregation. The ACO can collect data from most of the participant TINs. The small practices are unable to submit data to the ACO in the format needed to enable the de-duplication and aggregation steps that ACOs must complete before submitting a file to CMS, because the vendor system used by them will charge an additional fee to support the eCQMs on which the ACO must report that they cannot afford. In addition, one practice changed vendors midyear and as a result is unable to produce the needed files for the reporting year. The ACO is not able to determine the number of individuals who could be included in the eCQMs' eligible populations, so the ACO can either estimate the data completeness and report the measure without data from these practices or remove them from the ACO.

Furthermore, physicians are being held to a higher bar than any other CMS quality program. For example, health plans report on a sample of patients for each of the measures that require clinical data beyond administrative claims in the Medicare Part C and D Star Ratings. Hospitals also abstract clinical data on a sample of patients for the clinical process of care measures. None of these sample sizes, which are based on the number of plan participants or individuals admitted to the hospital for a specific diagnosis or procedure, comes close to the current 75 percent data completeness requirement in MIPS**. If CMS determined that smaller sample sizes provide sufficient information on which CMS and others can make informed decisions on the quality of care delivered for health plans and hospitals, this same logic should also apply to MIPS.**

Until physicians and other eligible clinicians can work within an environment where data and care are integrated seamlessly across settings and providers, it is premature to continue increasing data completeness and encourage reporting through a registry or EHR (or require eCQMs/MIPS CQMs under MSSP). Current policy levers such as MIPS Promoting Interoperability requirements or Information Blocking regulations cannot alone resolve data completeness issues. Technology, standards, costs, and implementation decisions made by CEHRT developers will continue to impact the completeness of quality reporting. As previously stated, varying interpretations and assumptions about policy play a key role. Therefore, we urge CMS to work with physicians and developers to solve the data completeness factors we have outlined.

> *(a) What would be the benefits and downsides of using Bulk FHIR data exports from EHRs to CMS to simplify clinical quality data submissions? Can CMS reduce the burden on providers by performing quality metrics calculations leveraging Bulk FHIR data exports?*
>
> *(b)In what ways can the interoperability and quality reporting responsibilities of providers be consolidated so investments can be dually purposed?*

Moving to a? the? FHIR based standard will allow physicians to incorporate alternative sources of data into quality reporting and should ease reporting burden. Specifically, it will allow specialties like radiology and gastroenterology to use their digital data sources. FHIR will also eliminate the need for the Promoting Interoperability (PI) category within MIPS because the use case is built into the quality measure(s) and automatically demonstrating the use of the EHR and other digital sources of data and tools. **Eliminating PI would also reduce vendor integration costs and practice reporting burden. However, CMS must provide sufficient time and avenues by which physicians and practices can successfully transition to dQMs.**

The transition will require both vendor readiness and additional education and resources for those reporting to understand and implement what is required. Practices must be knowledgeable of the current and future capabilities and technologies of their EHRs and other sources of digital data. While progress has been made, **this shared understanding is not yet at the level where most practices could move to FHIR-based reporting quickly, and we anticipate that all players in this process will need more time than the proposed two years outlined in the 2026 Inpatient Prospective Payment System Proposed Rule.**

Therefore, **we recommend CMS to focus on the transition to FHIR dQMs rather than initially just FHIR eCQMs, and as a result, we will be further in our journey to digital data and reduced data collection and reporting burden than the proposed phased approach.** Because we view FHIR eCQMs as a component of FHIR dQMs, entities such as hospitals or practices that may not be ready to use additional digital data sources would still be able to leverage primarily EHRs data while others, such as ACOs and health plans, could use additional sources available to them (e.g., administrative claims, labs, health information exchange data). Broadening to FHIR dQMs will provide the flexibilities that would allow groups to tailor their efforts based on where they are in collecting and reporting digital data and still shift to the optimal standard for data exchange.

We urge CMS to create and release a detailed timeline with milestones that indicate when critical steps and activities have been achieved and what factors and deliverables must be met to indicate that the field is sufficiently ready to move to the next step. For example, this process and timeline could outline when the technical requirements for FHIR-based reporting will be made available with adequate time for vendors to integrate them into their products and when these requirements will be incorporated into certification requirements. At the same time, CMS should also build the internal capabilities needed to receive this data through FHIR-based APIs and release guidance and education to assist practices and ACOs in this transition. Subsequently, once CMS determines that vendors are ready and certified to support this reporting and CMS can receive the data, a reasonable timeframe during which practices, hospitals, ACOs, and others must begin reporting these measures to CMS should be proposed.

This glidepath must include appropriate positive incentives to support practices, particularly small and rural groups, through each step of the transition in a thoughtful way. By using a stepwise approach with initial activities focused on building the required infrastructure, followed by data collection and reporting by the practices, all can be successful. It will be essential for each step to provide adequate time and resources, and a critical component will be an evaluation of when we are ready to move from one step to the next with input from the community on whether there is agreement that most if not all participants are ready to successfully report FHIR dQMs.

> *Can CMS reduce the burden on providers by performing quality metrics calculations leveraging Bulk FHIR data exports?*

Based on the current state of Bulk FHIR, the benefits are limited due to the well-documented barriers in its implementation. There are known performance limitations, inconsistencies in support across vendor implementation and technical barriers that CMS, HL7 and others must address to allow this standard to work as intended. Processing large datasets with Bulk FHIR can be slow, and the infrastructure required to mitigate this issue can be costly to setup and maintain—a general issue with FHIR. When a Bulk FHIR query fails, the required restart can add additional time and validation complexity, exacerbating the performance issues. The inability to change existing export groups results creates even more challenges when a B ulk FHIR query fails, requiring either a sophisticated code to ensure non-duplicative results or a complete re-run of the query.

It should also be noted that security and permission setup is non-trivial and if done incorrectly, may result in vulnerability at either end of the FHIR transaction. Organizations are becoming more risk-averse to moving patient-level data across firewalls. **While improved Bulk FHIR implementation makes that movement easier, additional regulatory attention around security and proper use would be required to gain compliance from EHRs, health systems, and practices.** Health systems rely on internal quality teams to implement continuous quality improvement programs, and small practices will likely not have many staffing and financial resources to allow them to quickly shift to this standard. Simply changing the reporting mechanism to Bulk FHIR for CMS quality measure reporting would not necessarily provide efficiency benefits or reduction in burdens to health systems and practices working to make quality changes at the point of care. Quality teams would need to be engaged to determine how the data can be leveraged to improve programs and represent the quality of care provided. Therefore, we recommend CMS develop solutions to the downsides of using Bulk FHIR data exports from EHRs to CMS to simplify clinical quality data submissions.

Furthermore, how FHIR endpoints are built, maintained, and mapped vary between EHRs, requiring, at minimum, custom setup to connect to each practice or vendor system. The additional work required by the practices should not be minimized as it will take time and resources to ensure that the mapping is valid. Additional setup and complexities can arise depending on who maintains the FHIR endpoint, and how much support is dedicated to maintaining it. EHRs are not incentivized to support Bulk FHIR, increasing the prevalence of the issue.

3. Digital Identity

PR-9 & PR-10. How might CMS encourage providers to accept digital identity credentials (for example, CLEAR, ID.me, Login.gov) from patients and their partners instead of proprietary logins that need to be tracked for each provider relationship? a. What would providers need help with to accelerate the transition to a single set of trusted digital identity credentials for the patient to keep track of, instead of one for each provider? b. How might CMS balance patient privacy with convenience and access to digital health products and services that may lead to significant improvements in health?

A single, high-assurance digital identity credential—such as ID.me, Login.gov, or CLEAR—could markedly improve patients' experience by replacing the patchwork of practice-specific usernames and passwords with one National Institute of Standards and Technology (NIST)-aligned credential that works across portals, telehealth apps, and TEFCA exchanges. This "one-and-done" proofing model would reduce forgotten-password churn, speed onboarding, curb record mismatches, and strengthen fraud protection, all while giving physicians greater confidence that the person requesting data is the right patient. **To accelerate uptake, CMS should certify a shortlist of identity assurance level 2 (IAL2)-compliant vendors, embed choice of credential into the existing FHIR-based API framework, and fund pilot programs that demonstrate seamless log-in within common EHR workflows—showing clinicians that the technology can enhance access without added clicks.**

Yet mandating digital IDs through payment incentives, or other CMS levers, would repeat the pitfalls of past federal health IT programs. Integrating third-party identity services requires new contracts, EHR customization, security audits, and staff training—costs that can quickly reach six figures for small, medium, or rural practices already strained by administrative cuts. Physicians would also shoulder inaccuracy and privacy risks: facial-recognition error rates are higher for some populations, and patients wary of biometric databases will demand parallel, manual workflows that erode efficiency gains. **CMS should therefore pair any identity initiative with vendor-neutral grants, regional technical-assistance centers, and hardship exemptions, keep adoption voluntary during a multi-year ramp-**

up, and prohibit punitive penalties tied to metrics outside physicians' control. Balancing convenience with privacy means protecting patient choice and shielding practices from unfunded mandates while the ecosystem matures.

4. Information Blocking

PR-12. Should ASTP/ONC consider removing or revising any of the information blocking exceptions or conditions within the exceptions (45 CFR part 171, subparts B through D) to further the access, exchange, and use of electronic health information (EHI) and to promote market competition?

The AMA appreciates CMS and ASTP/ONC's interest in information blocking exceptions. While there are instances where certified health IT developer actors misuse exceptions, and those developers should be investigated and penalized, the AMA is worried that any reduction in exceptions for one actor type would reduce the available exceptions for all actor types. Said another way, currently every actor can use any exception. Removing one or more exception would impact every actor, not just those who have abused exceptions.

The AMA strongly urges CMS and ASTP/ONC to retain every existing information-blocking exception—including Preventing Harm, Privacy, Health IT Performance, and Manner—because each one addresses a distinct, high-stakes scenario that physicians confront daily. For example, the Preventing Harm Exception lets a physician, based on the patient preference and choice, delay releasing a new Stage-IV cancer diagnosis until the patient can be counseled in person, averting documented risks of self-harm. The Privacy Exception permits withholding behavioral-health records when a patient affirmatively requests additional protection or where stricter state laws apply, thereby preserving patient trust and legal compliance. The Health IT Performance Exception allows temporary downtime for EHR patches and security upgrades—critical after the recent wave of ransomware attacks—while the Manner Exception (often misunderstood as merely a developer shield) lets a small practice furnish data via a certified API rather than an ad-hoc bulk export it lacks the tools to produce, ensuring exchange occurs without crippling local resources.

Eliminating or narrowing any exception would have system-wide collateral damage because all actors, not just the few bad-faith vendors, would lose the same safeguards. Physicians already wrestle with complex rule interpretation and fear punitive disincentives; shrinking the exception set would force over-disclosure of sensitive data, spike liability, and saddle small and rural practices with unfunded technology retrofits they cannot afford. Crucially, Office of the Inspector General (OIG) complaint data show no pattern of physicians abusing these exceptions: most investigations target developer actions outside clinicians' control. CMS' own disincentive rule further illustrates that technical limitations, not clinical intent, often trigger alleged blocking. Retaining the full suite of exceptions, coupled with clearer guidance and education, not additional penalties, is therefore essential to sustain trusted information exchange, protect patients, and avoid compounding the administrative and financial strain on a profession already facing payment cuts and escalating cybersecurity threats.

Payers

PA-5. What are ways payers can help with simplifying clinical quality data responsibilities of providers?

There must be alignment at the data element level, not just the measure concept.

a. *How interested are payers and providers in EHR technology advances that enable bulk extraction of clinical quality data from EHRs to payers to allow them to do the calculations instead of the provider-side technology?*

It is imperative that we streamline the prior authorization process to protect patients' access to timely care and reduce administrative burdens and waste for physician practices. One way to do so would be to simplify and standardize prior authorization criteria to minimize variation across plans. Studies have found wide variation between major health plans' PA requirements and rules, with this lack of uniformity contributing to poor transparency, practice hassles, and physician burnout. As proposed at your Senate confirmation hearing, CMS could, among other things, standardize the services subject to prior authorization and the associated clinical criteria to facilitate implementation of an end-to-end electronic process.

CMS has taken important steps toward such automation for Part D and Medicare Advantage plans, and the AMA is advocating for such standardization in electronic prior authorization across all plans. **We urge CMS, ASTP/ONC, and HHS to go a step further and require transparency and standardization around what services require prior authorization, the criteria used, and the data elements required by payers;** doing so would go a long way in simplifying and streamlining the process for physicians. In addition, this cross-payer standardization would reduce the need to digitize many thousands of proprietary payer prior authorization rules, which has been a rate-limiting factor in API implementation to date.

We caution CMS and ASTP/ONC about allowing private payers to perform bulk data extraction in real time from physician practices. While it might appear like a lofty goal to ease administrative burden, private payers do not have the best interest of patients at heart, and their overall goal is revenue based, as opposed to true quality improvement. We are also concerned it could violate patient privacy and diminish the doctor-patient relationship because bulk data extraction requires pulling identifiable patient information.

b. *In what ways can the interoperability and quality reporting responsibilities of providers to both CMS and other payers be consolidated so investments can be dually purposed? Are there technologies payers might leverage that would support access to real time quality data for healthcare providers to inform clinical care in addition to simplifying reporting processes?*

We recommend CMS build automated validity and reliability check tools for data verification to simplify reporting, but we do not envision the tools ever fully removing the need to have a person manually review the data to ensure accuracy.

One such example of a potentially helpful tool CMS developed is the de-duplication tool for ACOs (DeDupliFHIR). However, we have heard from practices and vendors that they are hesitant to utilize the

tool because it is downloadable from GitHub, an unsecure public website that puts them at risk of a data breach and exposing identifiable patient information.

PA-7. How can CMS encourage payers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal? What would be the impact? Would it advance or negatively impact data exchange?

The AMA shares CMS' commitment to eliminating true information blocking and advancing secure, patient-centered data exchange. We recognize the pivotal role payers can play in promoting interoperability and welcome efforts that hold **all** actors accountable while safeguarding privacy and clinical autonomy. Yet, the AMA is concerned that payer-driven information-blocking accusations could create significant unintended consequences.

CMS should **refrain from creating financial or regulatory incentives that spur payers to file information-blocking complaints against physicians.** Payers already wield significant contractual leverage and, as AMA surveys show, routinely threaten network status or tighter prior-authorization rules to force blanket EHR access, far beyond HIPAA's requirement to disclose only the minimum necessary information for any disclosures made to a person or entity who is not the patient. Giving payers a new, penalty-driven mechanism would invite abuse: complaints would become a negotiating tactic to extract entire medical records, eroding patient privacy, amplifying prior-authorization burden, and chilling open clinical documentation. ASTP/ONC's portal was designed to surface genuine, egregious blocking; transforming it into a payer grievance channel would flood the system with low-value disputes, divert oversight resources, and undermine provider trust in federal interoperability efforts.

Instead, **CMS should keep payers focused on data requests that are demonstrably tied to payment or narrowly defined health-care operations, consistent with longstanding HIPAA regulations.** Educating payers on the minimum-necessary requirement for any disclosures made to a person or entity who is not the patient, and enforcing existing privacy safeguards will do far more to advance secure, meaningful data exchange than weaponizing the complaint process. In short, empowering payers to police physicians will not accelerate interoperability—it will entrench adversarial dynamics, expand administrative waste, and ultimately harm patients by siphoning clinician time away from care.

Technology Vendors, Data Providers, and Networks

3. Technical Standards and Certifications

TD-5. How could a nationwide provider directory of FHIR endpoints improve access to health information for patients, providers, and payers? Who should publish such a directory, and should users bear a cost?

Accurate directories are critical instruments that facilitate patients' access to care. A nationwide provider directory that includes FHIR endpoints could be a valuable tool for physicians, health plans, and industry stakeholders to resolve this long-standing issue. **It is imperative, however, that any new directory does not create yet another place for physicians and practices to submit and update data adding to already significant administrative burden and taking time away from patient care.**

We see the value in provider directories that include FHIR endpoints and digital identity credentials, but it is important to recognize that the data required in directories is nuanced and largely driven by individual

contracts between providers and health plans, making data collection and maintenance complicated. The AMA collaborated with the Council for Affordable Quality Healthcare (CAQH) to examine and identify challenges associated with provider directory accuracy. In this report, **we recommended that health plans streamline the methods by which they request practices to submit data, align on fewer and more consistent update channels, and create a clear and concise workflow for updating this information to benefit all parties with greater predictability and opportunities for automation.**

Following these processes can lead to additional standards-based system-to-system integration that allows bulk and real-time updates and opportunities to harmonize data systems and eliminate data siloes. Moreover, we have advocated that practices should provide timely updates to health plans and health plans should reflect updates from clinicians and practices within their systems and directories in a timely manner. **We encourage CMS to work with the industry in a national effort to understand and solve for root cause issues that lead to directory inaccuracies and burden that are likely to continue, unless addressed.**

The AMA also supports the exploration of the use of digital identity technology to access and update information. This technology may aid in protecting against fraud and keeping this information secure.

The AMA urges more efforts along these lines to improve directory accuracy and ultimately the patient experience. Better coordination and information sharing will enable upstream improvements that minimize the administrative burden for all parties. The AMA welcomes the opportunity to share more detailed information with CMS and to participate in this effort.

We recognize the value in a nationwide provider directory of FHIR endpoints and digital identity credentials. However, this endeavor will need to build on the lessons learned from previous efforts and ensure that the creation of a directory is straightforward for physicians, helpful to patients, and burden-free for all.

TD-9. Regarding certification of health IT: (e.) *How could EHRs capable of bulk data transfer be used to reduce the burden on providers for reporting quality performance data to CMS? What capabilities are needed to show benefit?*

A simplified and more efficient Bulk FHIR process of clinical quality data submission would benefit health care organizations broadly and provide significant advantages for resource-limited health care organizations, as well as provide general interoperability benefits. Employing an exchange of patient-level information from an EHR to CMS could simplify data submissions by minimizing the analytics requirement on reporting departments and health systems. In addition to simplifying quality measure reporting, a well-established and reliable deployment of Bulk FHIR has value across population health, clinical research and public health surveillance. Ultimately, **the benefits of Bulk FHIR are functionally dependent on its level of maturity, ease of use, and the degree of standardization and mapping of clinical concepts across FHIR resources and EHR endpoints.** However, there appears to be an inherent assumption by CMS that if a physician or practice utilizes CEHRT, the EHR can support all quality measures, which is typically not the case.

CMS and ASTP/ONC need a process to ensure that when a practice purchases an EHR, either that EHR can support the practice's quality reporting needs, or it provides a way for the practice or the EHR vendor to seamlessly build in the additional need, without the vendor charging an additional cost to the practice. Currently, any addition a practice requests of an EHR vendor, including incorporating quality measures, is an additional cost. Therefore, **transition to Bulk FHIR requires CMS and ASTP/ONC to re-look at**

CEHRT and associated requirements so it can better support the needs of quality and provide assurances to physician practices that their CEHRT can support CMS quality reporting requirements.

It is also imperative that CMS ensure that measure specifications are aligned and vendors are supporting the measures for CMS quality programs. Recently, ACOs began reporting that some EHRs vendors were not supporting the Breast Cancer Screening eCQM since it had previously been removed as an individual measure option from MIPS. Practices that used these vendor systems are unable to produce the data needed for this eCQM and this lack of availability impacts not only reporting for Medicare Shared Savings Program (MSSP) but also for any physician or practice that elects to report this measure for the Focus on Women's Health MVP. Even if a vendor includes this reporting capability, there is an additional discrepancy since the eCQM specification was not updated to reflect the USPSTF's most recent recommendation to screen women aged 40-74 years of age; instead, the eCQM is limited to women aged 50-74 years of age. The MIPS CQM, however, was updated, leading to two specifications for the same clinical concept to include different age ranges and the associated benchmarking across collection types differ. **CMS must avoid situations where specifications are not clearly supported by vendors and specifications and associated benchmarking are not consistent.**

What concerns are there with this approach?

A fully functional Bulk FHIR framework has the potential to reduce burden on providers for reporting quality performance data to CMS. Physicians would have less requirements for analytical work around measures builds, testing and data auditing. Theoretically, fully functional Bulk FHIR implementation would also require a low level of additional interactions with a physician's EHR vendor to facilitate the Bulk FHIR transfer—set up of the outbound transfer. However, for it to be meaningful Bulk FHIR must provide reliable information back to the physician and practice in a timely manner, including performance scores on measures. **Therefore, there is additional work that must be performed to confirm that accurate and consistent data, information, and calculations can be provided back to physicians.**

We also have substantive concerns regarding the security of health information and adherence to minimum data use principles with bulk FHIR implementation. It is essential to establish a detailed outline of this work, including timeline, roles, responsibilities, and a clear path forward. **The following questions must be clarified and answered to ensure a smooth transition and implementation and gain buy-in from the physician community:**

- What are health systems and practices required to do? What expertise is needed?
- How does CEHRT support or not support? What is required due to CMS policy but not supported by CEHRT?
- Through what mechanism will data be sent to CMS? Where will it be stored? What are the security provisions related to the storage?
- How will the quality measure calculations and resulting scores be validated?

4. <u>Data Exchange</u>

TD-13. What new opportunities and advancements could emerge with APIs providing access to the entirety of a patient's electronic health information (EHI)?

a. What are the primary obstacles to this?
b. What are the primary tradeoffs between USCDI and full EHI, especially given more flexible data processing capabilities today?

The AMA supports CMS' goal of harnessing modern FHIR APIs to drive longitudinal care coordination, public-health reporting, quality measurement, and AI-enabled clinical decision support. Exposing *all* electronic health information (EHI) through a single endpoint might, in theory, streamline data aggregation and reduce custom interface work. Yet the same benefits can be realized—more safely—by continuing to expand USCDI on a predictable cadence, paired with clear implementation guides and a voluntary "sensitive-data" flag within FHIR that lets patients and physicians control granular sharing.

Mandating full-EHI (and write-back) APIs today would create steep technical and legal headwinds. Physicians would face new developer-initiated interface fees, complex data-segmentation builds, downstream-misuse liability, and consent-management workflows to comply with HIPAA—which bars disclosure of certain protected information without a signed attestation—and a fast-growing patchwork of state consumer-health privacy statutes in California, Connecticut, Nevada, and Washington. Allowing third-party apps—or payers—to *write* back to EHRs further magnifies risk: provenance becomes opaque, low-quality can corrupt the clinical record, and poorly secured APIs offer fertile ground for cyberattacks and AI data poisoning, as ASTP/ONC's own API-Write-Back workshop and recent security analysts warn. Requiring indiscriminate read/write access would chill documentation, heighten privacy exposure, and divert scarce resources away from patient care. CMS should therefore: **(1) keep USCDI as both the floor and practical ceiling for required API exchange; (2) fund pilots that test opt-in full-EHI and write-back capabilities with robust segmentation and provenance tracking; and (3) accelerate—but not leapfrog—USCDI expansion so innovation advances in lockstep with privacy, security, and real-world implementation capacity.**

5. <u>Compliance</u>

TD-18. Information blocking:

1. *Could you, as a technology vendor, provide examples for the types of practices you have experienced that may constitute information blocking. Please include both situations of non-responsiveness as well as situations that may cause a failure or unusable response?*
2. *What additional policies could ASTP/ONC and CMS implement to further discourage healthcare providers from engaging in information blocking practices?*

The AMA fully supports CMS' intent to root out true information blocking across **all** actor types and appreciates the focus on gathering concrete evidence of harmful practices. From the physician vantage point, the most persistent barriers originate with some health-IT developers: five- and six-figure "custom interface" fees to enable certified FHIR APIs, refusal to activate those APIs absent broad data-use waivers, PDF "data dumps" instead of discrete data, sluggish HL7 v2 feeds that stall connections to specialty registries, and months-long delays in turning on payer-requested interfaces. These non-responsive or unusable responses leave clinicians—and their patients—without timely, structured information, illustrating that technology-vendor conduct, not provider reluctance, often drives real-world data bottlenecks.

Given that CMS has yet to demonstrate that its newly finalized provider disincentives have been applied and/or improved exchange, piling on additional provider-focused penalties is unwarranted. Instead, CMS and ASTP/ONC should: **(1) pause any new punitive measures and redirect enforcement toward developer practices that demonstrably obstruct API functionality; (2) substitute education, corrective-action plans, and modest positive incentives for first-time findings—approaches HHS already uses in other quality programs—while ensuring penalties are proportionate and not**

duplicative across programs; and **(3) establish a single, timely appeals pathway so physicians can contest OIG determinations and CMS disincentives before financial hits occur, closing the due-process gap identified in the AMA's disincentive comments.** Without evidence of widespread provider actor misuse—and with no practical mechanism today for physicians to defend themselves—additional sanctions would simply magnify administrative burden, divert resources from patient care, and further discourage participation in Medicare quality and value-based initiatives.

Value-Based Care Organization

1. Digital Health Adoption

VB-1. What incentives could encourage APMs such as accountable care organizations (ACOs) or participants in Medicare Shared Savings Program (MSSP) to leverage digital health management and care navigation products more often and more effectively with their patients? What are the current obstacles preventing broader digital product adoption for patients in ACOs?

The AMA supports policies that promote equitable access to digital health tools and ensure that technology enhances, rather than burdens, clinical care. A coordinated federal strategy that aligns incentives, standards, and support mechanisms is critical to accelerating digital transformation in value-based care.

To encourage APMs to adopt digital health management and care navigation tools in support of patients, financial incentives aligned with quality outcomes and patient engagement metrics are essential. **CMS should offer enhanced shared savings or bonus payments for demonstrable use of certified digital tools that improve care coordination and patient outcomes.** Flexibilities should be built into policies and regulation that allow APMs to innovate in deploying digital health tools to patients. In addition, technical assistance and funding for infrastructure development, particularly for smaller or rural ACOs and their physician practices, can reduce barriers to entry for those entities that lack technical expertise and do not have resources to devote toward hiring outside consultants.

Current obstacles include interoperability challenges, lack of standardized data exchange, and limited integration of patient digital tools into clinical workflows. Many ACOs face resource constraints that hinder investment in advanced technologies, and inconsistent patient access to broadband and digital literacy gaps also limit the effectiveness of digital engagement strategies. Privacy and security concerns also deter broader adoption of digital health products, especially when third-party developers are involved who are not HIPAA-covered entities and do not abide by HIPAA's privacy and security requirements.

Another important consideration is that the ability to meet patients' data sharing preferences has been largely managed by "humans in the middle." That is, records requests have often been reviewed and considered by medical office staff. Sharing information with an outside entity would require information management professionals to manually collate and redact information, based on patient preference, law, or office policy.

To help address this gap, the AMA co-founded the Shift Collaborative for the purpose of advancing safe and patient-empowered sharing of health information for all. The Shift Collaborative is an independent health care task force of over 250 expert stakeholders that seeks to further a health care ecosystem in which better technology for clinical data sharing increases trust between patients, providers, and caregivers to improve health quality and outcomes.

Today's health data systems often force patients to choose between care and privacy. Patients should be able to share the information they want safely, securely, and on their own terms. Ensuring that individuals' data sharing preferences are met is a critical component of advancing safe, effective, and patient-centered value-based care and chronic disease management.

VB-2. How can key themes and technologies such as artificial intelligence, population health analytics, risk stratification, care coordination, usability, quality measurement, and patient engagement be better integrated into APM requirements?

To better integrate these technologies into APM requirements, APM EHR and technology utilization criteria should be sufficiently flexible to recognize the adoption of tools that support proactive, data-driven care management, particularly those that enable real-time identification of high-risk patients that may require enhanced services. **The AMA recommends that APM requirements include standards for usability and physician-centered design principles to reduce burden and enhance workflow integration.**

In terms of specifics, AI and analytics tools must be transparent, evidence-based, and subject to clinical oversight to ensure safety and equity. Incidentally, the AMA is committed to ensuring that AI can meet its full potential to advance clinical care and improve clinician well-being. As the number of AI-enabled health care tools continue to grow, the AMA has developed new policy that addresses the development, deployment and use of health care AI that should serve as a good reference when considering how best to integrate AI into APM requirements.

In addition, population health platforms should be interoperable with EHRs and public health data systems to support longitudinal care and community-level interventions. **Moreover, care coordination technologies should facilitate seamless transitions of care and bi-directional information exchange across settings.** Care coordination technologies will also be helpful to CMS in evaluating which models are most effective and should be scaled for further development.

For patient engagement tools (including mobile apps and devices for patient self-management of their conditions), there needs to be a broad evaluation process completed for accessibility, cultural relevance, and digital literacy. Quality measurement should evolve to include patient-reported outcomes and real-time feedback mechanisms. APMs present an opportunity to pilot solutions that test the integration of these technologies in diverse clinical settings, with a focus on scalability and equity. **It is imperative that CMS embrace this opportunity by implementing flexible policy requirements and providing more technical and financial support up front, which is especially important to ensure that smaller or resource-constrained physician practices can adopt and benefit from these innovations and participate in APMs.**

VB-3. What are essential health IT capabilities for value-based care arrangements?

a. *Examples (not comprehensive) may include: care planning, patient event notification, data extraction/normalization, quality performance measurement, access to claims data, attribution and patient ID matching, remote device interoperability, or other patient empowerment tools.*
b. *What other health IT capabilities have proven valuable to succeeding in value-based care arrangements?*

Essential health IT capabilities for value-based care arrangements must support proactive, coordinated, and data-driven care delivery. Core functionalities include care planning tools that enable shared decision-

making and longitudinal tracking of patient goals and interventions across diverse care settings and IT systems. Patient event notification systems are also critical for timely care transitions and reducing avoidable readmissions. Data extraction and normalization capabilities ensure that disparate data sources can be harmonized for analytics, reporting, and clinical decision support. In addition, quality performance measurement tools must be integrated into clinical workflows to reduce administrative burden and support real-time feedback.

The AMA has advocated that, as CMS looks to bridge the gap between MIPS and APMs, increasing data sharing and performance feedback is paramount for practices to monitor their performance and build confidence to move into APMs. Interoperable health IT tools can also help ensure that physicians have the most relevant, up-to-date information available on their patients to support real-time care decisions.

It is important to note that access to claims data, attribution of patients, and patient identity matching are also foundational for accurate performance evaluation and population health management.

Additional valuable capabilities for value-based care arrangements include remote device interoperability and patient empowerment tools, such as mobile health apps and patient portals, that enhance engagement and self-management. Technologies that can ease the burdens on patients and make their care journeys more trouble-free should be systematically piloted in real-world settings to determine effectiveness before being fully rolled out.

The AMA emphasizes that these technologies must be interoperable, standards-based, and designed with clinician input (the AMA can be helpful here) to ensure real-world usability and to guarantee trust in the technologies. Investment in these capabilities should be supported by federal funding, technical assistance, and alignment with certification and reporting requirements.

VB-4. What are the essential data types needed for successful participation in value-based care arrangements?

Successful participation in value-based care arrangements requires access to a comprehensive set of standardized, high-quality data types. Clinical data from EHRs, including diagnoses, procedures, medications, lab results, and vital signs, form the foundation for care coordination and quality measurement. Claims and administrative data are also extremely valuable to physicians and provide insights into utilization patterns as well as cost trends and need to be integrated at the point of care with other data sources. Social determinants of health (SDOH) data are increasingly vital for risk stratification and tailoring interventions to address non-clinical drivers of health outcomes. Patient-reported outcomes and care experience measures also offer critical perspectives on quality and effectiveness from the patient's viewpoint.

In terms of other data types, behavioral health data, often siloed from the rest of a patient's record, must be integrated to support whole-person care. Also, the ability of specialty physicians, such as gastroenterologists and radiologists, to incorporate their digital data sources into health IT systems is critical for participation in value-based care. Real-time data from remote patient monitoring and wearable devices can also enhance chronic disease management and present opportunities for early intervention. In addition, public health data, including immunization records and disease surveillance information, support population-level planning and response and need to be available at the point-of-care. **The AMA advocates for standardized data formats, with robust built-in privacy protections, and equitable access to ensure that data-driven, value-based care benefits all patients and physicians. To get the essential data types right will take time and constantly evolve as care delivery continues to improve**

in leveraging technology. The AMA, health IT developers, and other stakeholders can play a critical role in helping define these data types and should be called upon by CMS and ASTP/ONC.

2. Compliance and Certification

VB-5. In your experience, how do current certification criteria and standards incorporated into the ONC Health IT Certification Program support value-based care delivery?

The ASTP/ONC Health IT Certification Program provides a foundational framework that supports value-based care delivery by promoting interoperability, data exchange, and patient access. CEHRT requirements ensure that essential functionalities—such as support for electronic transitions of care, closing referral loops, and making the process for patients to get their personal health information less time-consuming—are available across care settings. These capabilities facilitate care coordination, quality measurement, and patient engagement, which are central to value-based models.

However, the AMA notes that certification criteria often lag behind the evolving needs of APMs, particularly in areas such as real-time data exchange, SDOH, and advanced analytics. The program's emphasis on structured data and standardized APIs, including the HL7 FHIR specification, is a positive step toward enabling more seamless data sharing. Yet, implementation variability and usability challenges continue to burden physicians and limit the full potential of certified tools, especially in small and medium-sized practices that lack health IT expertise and extensive technical infrastructure.

Certification should more explicitly support criteria that prioritizes physician-centered design principles and reduces documentation burden. Greater alignment between the ASTP/ONC Certification Program and CMS value-based care requirements would enhance the coherence of both programs and reduce complexity for physicians and other providers. We also emphasize that ongoing stakeholder engagement is essential to ensure that certification evolves in tandem with clinical and operational realities, especially those challenges facing small and medium-sized physician practices.

VB-6. What specific health information technology capabilities that could benefit APMs are not currently addressed by existing certification criteria and standards that should be included under the ONC Health IT Certification Program?

The AMA supports a certification framework that is flexible, scalable, and aligned with the practical needs of diverse APM participants, including both large and small entities. **We recommend expanding certification to include patient engagement technologies that support shared decision-making and remote patient monitoring.** Certification should also address data provenance and integrity to ensure trust when there are patient data aggregation opportunities. Smaller and rural providers would benefit from more modular certification options that allow incremental adoption of advanced capabilities. In addition, ongoing collaboration with the broader APM participant community is essential to ensure that certification evolves to support innovation in value-based care delivery without increasing burden on participating entities.

VB-7. How can technology requirements for APMs, established through CEHRT or other pathways, reduce complexity while preserving necessary flexibility?

The AMA has long advocated for ensuring that certification aligns with real-world clinical workflows and interoperability goals, especially for reducing provider burden through streamlined health IT certification requirements as well as supporting innovation by allowing modular certification of specific functionalities.

CMS and ASTP/ONC should adopt a modular certification approach that allows APMs to select capabilities aligned with their clinical focus and operational maturity. This approach would enable organizations to meet core requirements while tailoring additional functionalities to their specific care models.

In addition, streamlining reporting requirements and aligning them across programs would further reduce duplication and complexity. Certification pathways should also accommodate innovation by allowing for alternative methods of demonstrating compliance, such as real-world testing or pilot program participation. Smaller and rural providers should be granted extended implementation timelines and technical assistance to meet evolving requirements without needing to divert limited resources away from direct patient care. The AMA emphasizes that any flexibility in technology requirements must not compromise interoperability, data integrity, or patient safety. **We recommend that a collaborative governance model is used that involves physicians, other clinicians, developers, and policymakers to ensure that technology requirements remain responsive and practical.**

VB-8. How can other HHS policies supplement CEHRT requirements to better optimize the use of digital health products in APMs? As an example, requirements under the Conditions of Participation for hospitals (42 CFR 482.24(d)) require hospitals to transmit electronic patient event notifications to community providers. What barriers are in place preventing APM participants from receiving the same notifications?

A coordinated federal strategy across agencies and programs is essential to ensure that digital health tools enhance care quality and efficiency across all APMs. The AMA supports policy harmonization that enables APMs to receive not just mandatory data elements but all clinically relevant data—such as admissions, discharges, and transfers—at the point of care without the additional administrative burden of sorting through hundreds of pages of outdated or not directly relevant information, a practice commonly known as "data dumping."

Interoperability and data exchange issues also present barriers that prevent APMs from receiving these notifications. Known patient attribution is a relevant concern as well, since hospitals may not be informed that a particular patient is assigned to an ACO, and that organization needs to receive these notifications to be accountable for that individual's quality and cost of care. In addition, privacy concerns, lack of trust, and data-sharing agreements often delay or prevent the flow of information to APM entities.

HHS should address these challenges by further standardizing data-sharing protocols and promoting the use of APIs and FHIR-based solutions to facilitate real-time, patient-centered data exchange. We recommend that HHS provide funding and technical assistance to help smaller and rural APMs implement and sustain the IT infrastructure and optimize the capabilities of these products to benefit patient care, along with a longer glide path to meeting the full implementation requirements.

VB-9. What technology requirements should be different for APM organizations when comparing to non-APM organizations (for example, quality reporting, and interoperability)?

Technology requirements for all practices should prioritize interoperability, real-time data exchange, and advanced analytics to support population health management and value-based care delivery. These capabilities are especially important for practices participating in and preparing to transition to APMs. The AMA has repeatedly urged for timely (i.e., at least quarterly) MIPS performance data, which is

required under the Medicare Access and CHIP Reauthorization Act. Yet, MIPS performance data continues to be provided on an annual basis up to 18 months after their performance was measured. **This time lag makes it impossible for practices to use this data to improve clinical workflows and patient outcomes and prepare to transition to APMs.**

Quality reporting should also be fully integrated into clinical workflows, so that data can be captured in real-time and manual data entry is minimized. We support smaller APMs being granted phased implementation timelines and access to shared infrastructure to meet technology requirements. Remote patient monitoring, care coordination platforms, and patient engagement tools should also be supported for all practices, but particularly those in APMs, to manage high-risk populations and reduce avoidable care utilization. Overall, flexibility around technology requirements must be preserved to accommodate diverse APM models and the resources available in the provider's IT environment.

VB-10. In the Calendar Year (CY) 2024 Physician Fee Schedule final rule (88 FR79413), CMS established that CEHRT requirements for Advanced APMs beyond those in the "Base EHR" definition should be flexible based on what is applicable to the APM that year based on the area of clinical practice. What certification criteria should CMS identify under this flexibility for specific Advanced APMs, or for Advanced APMs in general? Are there specific flexibilities or alternatives to consider for smaller or resource-constrained (such as rural) providers in meeting CEHRT requirements without compromising quality of care or availability of performance data?

The AMA has previously been very vocal about more customized CEHRT criteria, however, we remain extremely concerned about CMS' requirements to increase the CEHRT utilization threshold to "all" eligible clinicians in an APM, which significantly increases administrative burden and will likely result in dropping or not recruiting practices that face CEHRT implementation barriers, which are disproportionately safety net, small, rural and other types of practices with fewer resources. **We strongly urge CMS to immediately reverse this requirement and revert to the previous standard of 75 percent of eligible clinicians, or to adopt a moderately higher threshold that is not 100 percent, leaving zero room for onboarding these types of practices to the ACO and using shared savings payments to invest in purchasing CEHRT.**

In terms of more flexible EHR Certification Criteria, the priority should be on functionalities that directly support the goals of specific APMs, such as care coordination, quality measurement, and patient engagement. For example, some APMs may require robust clinical decision support and genomics integration, while primary care models may benefit more from behavioral health integration and chronic disease management tools.

CMS should identify core certification criteria applicable to all APMs—such as interoperability, patient access, and data exchange—while allowing flexibility in advanced capabilities. **Smaller or resource-constrained providers, including rural physician practices, should be offered modular certification pathways, extended compliance timelines, and access to shared services or regional health IT-focused centers that could offer technical assistance.** Alternatives to meeting CEHRT requirements could include "yes/no" attestation-based reporting or targeted education training programs that focus on reducing administrative burden while preserving care quality and availability of performance data across the APM.

CMS staff had previously indicated it will engage with model participants and CMMI staff to design targeted CEHRT criteria on a model-by-model basis, yet the AMA has heard no updates since this policy

was finalized in November 2023. **The AMA supports this approach and encourages CMS to execute this plan with public transparency and stakeholder input.**

3. Technical Standards

VB-11. What specific interoperability challenges have you encountered in implementing value-based care programs?

Interoperability remains a significant barrier to the effective implementation of value-based care programs, particularly due to the lack of standardized data formats and inconsistent implementation of FHIR APIs across vendors and health systems. **We recommend the use of a minimum standardized data set and data elements to facilitate more meaningful and useful sharing of clinical and financial data.**

Addressing these challenges also requires stronger federal oversight of certified health IT developers as part of the enforcement of information blocking rules, investment in shared infrastructure, and alignment of standards across federal programs. Many physicians still encounter difficulties accessing external data due to proprietary interfaces or incomplete data sets. Patient identity matching across systems remains unreliable, leading to the fragmentation of patient records across providers and care gaps.

In addition, many organizations frequently operate outside of interoperable networks, such as behavioral health, social services, and long-term care providers, creating blind spots in the delivery of whole-person care. The AMA also notes that small and rural practices often lack the technical infrastructure or resources to participate fully in health information exchanges. Moreover, inconsistent consent management frameworks or varied state laws complicate data sharing, particularly for sensitive health information. These challenges undermine the goals of value-based care by impeding care coordination, risk management, and quality measurement.

VB-12. What technology standardization would preserve program-specific flexibility while promoting innovation in APM technology implementation?

Standardization efforts must focus on foundational interoperability as well as modularity to preserve program-specific flexibility and promote innovation in APM technology implementation. **The AMA supports the adoption of widely accepted standards such as HL7 FHIR, United States Core Data for Interoperability, and standardized APIs to ensure consistent data exchange across platforms.** These standards should be implemented in a way that allows APMs to customize workflows and tools based on their clinical and operational needs. Modular certification and plug-and-play interoperability protocols can enable innovation by allowing organizations to integrate best-in-class solutions without being locked-in to one vendor.

In addition, standardized data elements and exchange protocols reduce the burden of custom interfaces and facilitate more efficient scaling of digital tools. At the same time, flexibility in implementation timelines and reporting formats can accommodate the diverse capacities of APM participants, especially smaller or rural providers. **The AMA recommends that ASTP/ONC and CMS provide clear guidance on core versus optional standards to help organizations prioritize technology investments.** Innovation sandboxes and pilot programs can also support the safe testing of emerging technologies within a standardized framework. Ensuring that standards are developed with input from physicians, other

clinicians, developers, and patients will enhance usability and adoption and lead to a balanced approach to standardization that can drive both innovation and quality in value-based care.

VB-13. What improvements to existing criteria and standards would better support value-based care capabilities while reducing provider burden?

To better support value-based care while reducing provider burden, **existing certification criteria and standards must be refined to emphasize usability and alignment with clinical workflows. Systems and technology must serve clinicians, not the other way around.**

As we previously discussed, the AMA recommends expanding the use of standardized APIs and FHIR-based tools to facilitate seamless interoperability and reduce custom integration costs. However, without an EHR certification requirement, many physicians—especially those in small and rural practices—will not be able to access new APIs or take advantage of their promising capabilities. Establishing a process to evaluate, test, and certify new API technology will create standard APIs, promote end-to-end interoperability between EHRs and payer systems, and allow for an electronic process that is meaningfully integrated within the EHR workflow while supporting value-based care delivery.

In addition to standards and criteria, there needs to be a focus on how systems are architected in order to minimize burden. For value-based care to function effectively and efficiently, digital quality measurement should allow for seamless quality reporting that reduces burden and provides real-time performance data that can be used to improve patient care. There should also be a focus on limiting the number of platforms and systems participating practices must interact with to access patient and performance data to minimize burden, ideally by maintaining quality and financial performance data in the same platform or application. We also encourage dynamic dashboards to be built in ACOs to display quality, cost, and utilization data with helpful contextual or comparative information and actionable data-driven insights for participating practices to easily and quickly monitor performance and identify areas for improvement.

VB-14. How could implementing digital identity credentials improve value-based care delivery and outcomes?

Implementing digital health credentials could one day improve value-based care delivery for both patients and physicians. These credentials can significantly enhance the delivery and outcomes of value-based care by ensuring accurate, secure, and efficient identification of patients and physicians across ACOs—reducing administrative errors, improving patient matching, and streamlining care coordination.

For patients, verified digital identities can improve their access to personal health information, enable consent management, and support engagement in their own care journey through digital tools. **However, as previously discussed, there are underlying privacy and security risks in creating centralized biometric data hubs with individuals' digital health credentials.** Public trust is already unsettled by recent reports of data misuse and breaches, and pushing adoption of credentials too aggressively will deepen that skepticism and slow, not accelerate, the uptake of digital health.

For physicians, these credentials can help facilitate secure access to IT systems and enhance accountability and trust in documentation as well as patient-centered data exchange. Integration with existing health IT infrastructure, including EHRs, and patient portals, would be essential for seamless adoption. Implementation in value-based care should proceed cautiously as integrating third-party identity

services requires new contracts, EHR customizations, security audits, and staff training—and costs can escalate quickly. There may be some economies of scale in implementing digital credentials across an ACO, but small and rural practices remain under strain as declining physician reimbursement rates impact physicians' ability to invest in their practices.

If CMS moves forward with implementing digital credentials in value-based care delivery, **we recommend that adoption be voluntary across ACOs or piloted in a small number of models to demonstrate value and prove effectiveness.** Physicians will only adopt digital credentials in value-based care when the technology proves it can protect patients, integrate seamlessly into clinical workflows, and remain affordable to physicians.

VB-15. How could a nationwide provider directory of FHIR endpoints help improve access to patient data and understanding of claims data sources? What key data elements would be necessary in a nationwide FHIR endpoints directory to maximize its effectiveness?

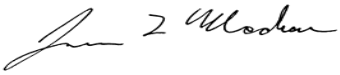
As previously discussed, accurate directories are critical instruments that facilitate patients' access to care. A nationwide provider directory that includes FHIR endpoints could be a valuable tool for physicians, health plans, and industry stakeholders to solve this long-standing issue. **It is imperative, however, that any new directory be created and maintained in such a way that it will not compound administrative burdens for physicians, as requiring each provider to submit and update data would add to already significant administrative burden and take time away from patient care.**
We see the value in provider directories that include FHIR endpoints, but it is important to recognize that the data required in directories is nuanced and largely driven by individual contracts between providers and health plans making data collection and maintenance complicated. The AMA's report with CAQH called on health plans to undertake several remedies to help improve provider directory accuracy, including the creation of a clear and concise workflow for updating this information to benefit all parties with greater predictability and opportunities for automation.

We encourage CMS to work with the industry to understand and solve for root cause issues that lead to directory inaccuracies and burden that have the propensity to continue to be propagated through a national effort.

The AMA urges more work to improve directory accuracy and ultimately the patient experience. Better coordination and information sharing will enable upstream improvements that minimize the administrative burden for all parties. We would welcome the opportunity to share more detailed information with CMS and participate in any nationwide provider directory effort.

Thank you for considering the AMA's recommendations. If you have any questions, please do not hesitate to contact Matt Reid, Senior Strategic Digital Health Policy Consultant, at matt.reid@ama-assn.org.

Sincerely,



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